

FOOTBALL PITCH-SIDE PRIMARY SURVEY PROTOCOL

1. Triage Framework & Protocol Selection

- **Definition:** A structured assessment and intervention process designed to identify life- or limb-threatening injuries and enable clinicians to perform evaluations efficiently.
 - **Protocol Selection:**
 - **Complete Primary Survey:** Involves a full systematic examination across all components (Airway, C-Spine, Breathing, Circulation, Disability, Exposure). It is indicated when the mechanism of injury is concerning or unclear, or when responses from the player, referee, or teammates raise clinical concern. Complete survey is safer, minimizes the risk of missing significant secondary injuries, and builds clinician fluency through repeated practice.
 - **Focused Primary Survey:** Examines only the specifically suspected injured body part. It is indicated when the mechanism of injury is clearly witnessed, the injury is isolated, responses are un concerning, the player confirms no other symptoms, and clinical instinct supports an isolated injury.
 - **Safety Transition Rule:** If any concern arises or if the player's condition changes, immediately switch to a complete primary survey.
- [See Figure 1: Initial Assessment Pathway \(next page\).](#)
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2. Pre-Survey Scan & Safety Checks

1. **PPE:** Don personal protective equipment prior to entering pitch or assessing.
 2. **Danger Assessment:** Check for immediate environmental hazards.
 3. **5-Second Rapid Scan:** Perform an immediate 5-second assessment looking for:
 - Signs of upper airway obstruction.
 - Central pulse.
 - Catastrophic haemorrhage.
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Choose the focused or complete primary survey based on mechanism and responses (see Section 1).

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graph TD; Start((Injury occurs)) -- Green --> GreenCriteria[• Mechanism clearly seen and injury is immediately felt to be isolated and unconcerning for an A or B injury  
AND  
• Player's response is clearly seen and is unconcerning  
AND  
• Referee and team-mate response is unconcerning  
AND  
• Clinician gives consideration to potential for A and B as approaching player]; Start -- Red --> RedCriteria[• Mechanism is concerning or unclear  
OR  
• Player's response is concerning  
OR  
• Referee or team-mate's response is concerning]; GreenCriteria -- Green --> GreenConfirm[• Player confirms no other symptoms or issue  
• No other concerns highlighted  
• Clinical instinct is of isolated injury]; RedCriteria -- Red --> RedSurvey[• Immediately proceed to protect cervical spine and assess player fully using A B C primary survey process]; GreenConfirm -- Green --> Final[• Assess the injury  
• Low threshold for going back to full primary survey]; RedSurvey -- Red --> Final; Change[change in condition] -- Red --> Final; Concerns[Any concerns or change of condition] -- Red --> Final;
```

Injury occurs

Clinician enters pitch

Green Path (Isolated Injury):

- Mechanism clearly seen and injury is immediately felt to be isolated and unconcerning for an A or B injury
- AND
- Player's response is clearly seen and is unconcerning
- AND
- Referee and team-mate response is unconcerning
- AND
- Clinician gives consideration to potential for A and B as approaching player

Red Path (Concerning Injury):

- Mechanism is concerning or unclear
- OR
- Player's response is concerning
- OR
- Referee or team-mate's response is concerning

Green Path Confirmation:

- Player confirms no other symptoms or issue
- No other concerns highlighted
- Clinical instinct is of isolated injury

Red Path Action:

- Immediately proceed to protect cervical spine and assess player fully using A B C primary survey process

Final Step (Both Paths):

- Assess the injury
- Low threshold for going back to full primary survey

Triggers for Return to Survey:

- change in condition
- Any concerns or change of condition

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3. Performing the Primary Survey (A , B, C, D, E)

A — AIRWAY & CERVICAL SPINE

- **Airway Evaluation:**
 - **Look:** Inspect for facial trauma, including wounds, swelling, hematoma, bleeding, deformity, or foreign bodies.
 - **Listen:** Listen for signs of upper airway obstruction (stridor, gurgling, snoring, choking, or hoarseness).
 - **Feel:** Feel for facial tenderness, swelling, or structural deformity.
 - **Quickest Method:** Speak directly to the player to quickly assess airway patency.
- **Airway Interventions (If Obstruction Present):**
 - **Manual Opening:** Perform a **jaw thrust** manoeuvre.
 - **Suctioning:** Suction bleeding, secretions, or vomitus while simultaneously evaluating the gag reflex with the suction catheter.
 - **Airway Adjuncts:** Insert an **Oropharyngeal Airway (OPA)** if the gag reflex is absent. Consider a **Nasopharyngeal Airway (NPA)** if a gag reflex is present.
 - **Oxygen Therapy:** Administer high-flow oxygen via a Bag-Valve-Mask (BVM) or Non-Rebreather Mask at **15 L/min**.
 - **Advanced Management:** Consider **Laryngeal Mask Airway (LMA)** insertion pitch-side in cardiac arrest scenarios, and **Rapid Sequence Intubation (RSI)** later at the medical base, ambulance, or hospital.
- **Cervical Spine Management:**
 - Initiate **Manual In-Line Stabilisation (MILS)** and palpate for cervical spine tenderness.
 - Early cervical collar application is not required if the player is conscious and alert, as alert players naturally splint their neck.
 - Apply a cervical collar if you are unable to clear the cervical spine.
 - Use the **NSAID** mnemonic to determine if the spine can be cleared on-field.

- **The NSAID Checklist:** If **any** of the following are present, the spine **cannot** be cleared and the player must be extricated in a collar:
 - Neurological deficit (numbness, weakness)
 - Spinal midline tenderness (on palpation)
 - Altered mental status (GCS <15)
 - Intoxication
 - Distracting injury (any injury severe enough to mask spinal pain)
- Player must also be able to actively move the neck 45° lateral rotation and the mechanism of injury is not concerning

Note: NSAID here is a spinal-clearance mnemonic, not the analgesic drug class.

B — BREATHING

- **Pitch-Side Limitations:** Pitch-side breathing assessment is limited and utilizes a focused "**hands-on 1, 2, 3 approach**".
- **Step 1 — Observe:** Evaluate respiratory rate (RR), chest wall movement, deformity, flail segments, bruising, and open wounds.
- **Step 2 — “2 Hands on 3 Places”** : Place hands sequentially across three key anatomical locations:
 1. Neck.
 2. Anterior chest wall.
 3. Posterolateral chest wall.
- **Assess:** For tenderness, swelling, crepitus, and deformity.
- Auscultation would have no value inside a noisy stadium

C — CIRCULATION & HAEMORRHAGE CONTROL

- **Initial Assessment:** Check for active haemorrhage and evaluate **CCTVR**.
- **Anatomical Scan "2 Hands on 3 Places":**
 1. **Abdomen:** Inspect and palpate across all 4 quadrants.
 2. **Pelvis:** Inspect, then perform gentle palpation in sequence: **posterior → lateral → anterior**. (DO NOT SPRING THE PELVIS)
 3. **Long Bones:** Inspect and palpate for structural injury.

- **Targeted Interventions:**

- Apply direct pressure to control active bleeding.
- Apply splints for suspected fractures.
- Utilize a scoop stretcher to minimize movement during handling.

- **Hemodynamic Checks:**

- **Capillary Refill Time (CRT):** Measure centrally by pressing the skin over the sternum.
- **Blood Pressure (BP):** On-pitch BP has no clinical value and should be evaluated later.

SBP Estimator: A palpable radial pulse indicates a minimum Systolic Blood Pressure (SBP) of approximately **80–90 mmHg**.

D — DISABILITY & NEUROLOGICAL STATUS

- **Level of Consciousness: AVPU (preferred) or GCS assessment**

- **Alert (A)** = GCS 15.
- **Voice (V)** = GCS 13.
- **Pain (P)** = GCS 8.
- **Unresponsive (U)** = GCS 3.
- Any assessment score other than **Alert** mandates **immediate removal from the field of play**.

- **Pupillary Evaluation:** Assess pupil size and reactivity to light.

- **Gross Neurological Assessment:**

- Rapidly scan for unilateral neurological deficits.
- Inquire about sensation changes (numbness or weakness).
- Observe limb movements and instruct the player to move their limbs.
- Assess balance and gait when returning the player to a standing position if appropriate.

E — EXPOSURE & EXTRICATION

- Complete necessary exposure and safely extricate the player from the pitch.

Extrication kit: trauma = scoop stretcher + basket, set up below the feet in line with the player (basket 2–3 lengths away).
Cardiac arrest = spinal board ± basket (see Station 5).